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ONLINE TRAINING · EPISODE 02

In-Office Whitening Workflow

Step-by-Step Protocol + Common Mistakes

Dr. Germán Gómez

DDS · MD · PhD (University of Tübingen) · Esthetic Dentistry & Implantology, Spain

500+ lectures · 43 countries



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HOOK

“My friend had her whitening done here — hers looks incredible. Will mine?”



THE HARD TRUTH

Same gel, same lamp — two clinics, visibly different results. The chemistry never changes between offices. The workflow does.

THE FLÄSH ANSWER

If the outcome depends on who happens to be working that day, you have a whitening lottery. A strict, standardized protocol ends the lottery.

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Predictable outcomes are engineered.

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PRINCIPLE · THE BIG IDEA

A Protocol, Not a Performance

01 A closed system.

The 32% gel, Highpower LED lamp and fläshcard guidance work as one unit.

02 One standard.

Same steps, same order, every patient — whoever is working today.

03 Happy patients, every time.

An engineering claim, not a slogan — once the human steps are standardized.

04 Effortless for the clinical team.

The lamp carries the physics, the card the clock — your team the patient.

PRINCIPLE · THE BIG IDEA

30

years of German-made whitening expertise

Gel, lamp and fläshcard engineered as one closed system · WHITEsmile



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PRINCIPLE · THE METHOD

The Five-Phase SOP

1 **Document**
Clean first, baseline shade in value order, standardized photos.

2 **Protect**
Vitamin E or vaseline, correct retractor, goggles, cured barrier.

3 **Apply**
A minimum 1 mm of green 32% gel on every facial surface.

4 **Activate & monitor**
Lamp locked on the retractor, fläshcard cycles, comfort checkpoints.

5 **Seal & instruct**
After Whitening Mousse, final documentation, the aftercare rules.



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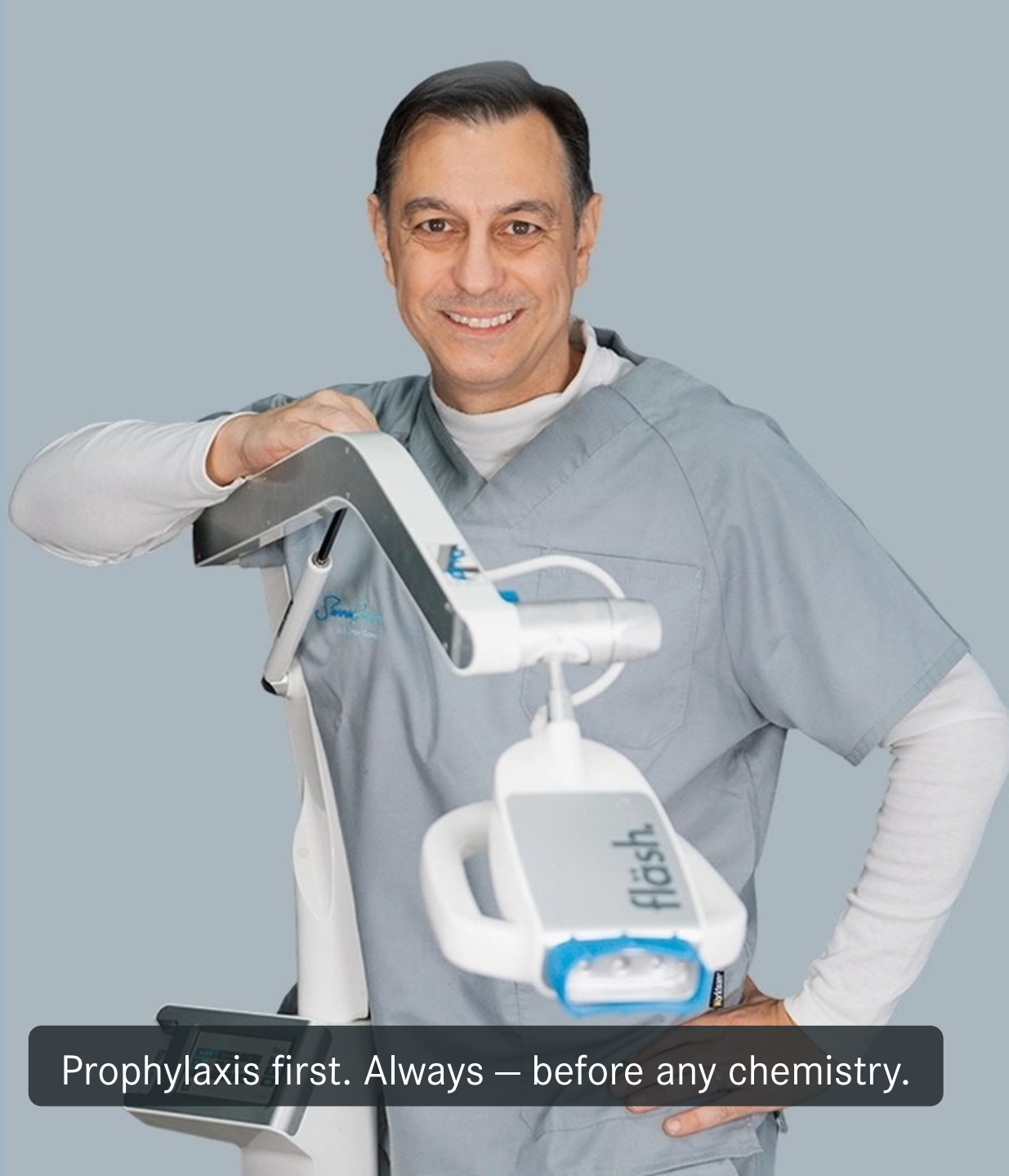
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PHASE 1 OF 5

Document

Whitening never begins on an uncleaned tooth.

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Prophylaxis first. Always — before any chemistry.

PHASE 1 · DOCUMENT

Clean Before You Whiten

01 Prophylaxis first.

Always — plaque, calculus and stain removed before any chemistry.

02 Cleaning can be enough.

If polishing delivers the wanted shade, say so — nothing builds trust faster.

03 Clean enamel, even contact.

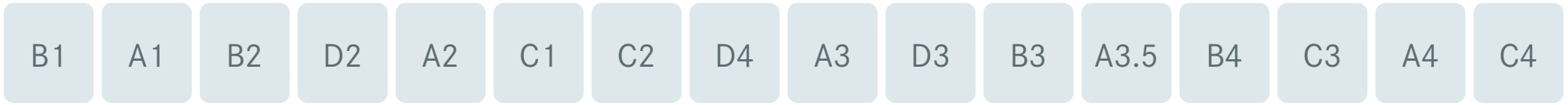
Your first defense against the uneven result in our mini case.



Professional prophylaxis — before and after

PHASE 1 · DOCUMENT

The Value-Ordered Baseline



Value-ordered shade guide · lightest to darkest

01 Determine it together.

Daylight, teeth still hydrated, the tab in the patient's own hand.

02 Enter it into the lamp.

After treatment it reports the shades gained — a number, not an opinion.

EVIDENCE

Joiner A., J Dent 2004

PHASE 1 · DOCUMENT

Photograph Like a Scientist

01 The identical setup.

Same camera, distance, angle and neutral light.

02 Motivation.

Objective progress, session after session.

03 Ethical marketing.

Consistent pairs, always with written consent.

04 Protection.

If an outcome is disputed, your records speak.

CLINICAL FOCUS

Consistency is credibility.

Identical setup, before and after.



2

PHASE 2 OF 5

Protect

Where every complication is born — and prevented.



Moisten the lips first; then the fläsh lip retractor, docked onto the lamp head.

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PHASE 2 · PROTECT

Soft Tissue First

01 Vitamin E or vaseline on the lips.

Then the fläsh lip retractor, docked onto the lamp head.

02 Filtered eye protection.

For the patient and for every team member present.

03 Gingiva protector, light-cured.

Dry gingiva; only slight overlap onto cervical enamel; cure until solid.



Lamp docked on the lip retractor · filtered goggles

EVIDENCE

Li & Greenwall, Br Dent J 2013

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PHASE 2 · PROTECT

The Seven Sins of Isolation

1. No lip protection — vitamin E or vaseline
2. Wrong retractor for the system
3. No filtered eye protection
4. Barrier not properly placed or cured
5. Not enough whitening gel (below 1 mm)
6. Patient left unattended — no signal agreed
7. Dentin sensitivity not managed

CLINICAL FOCUS

Burns are never caused by the gel.

Rescue: vitamin E or vaseline onto the gingiva — no panic, protocol.

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PHASE 3 OF 5

Apply

The gel is engineered for precision.

PHASE 3 · APPLY

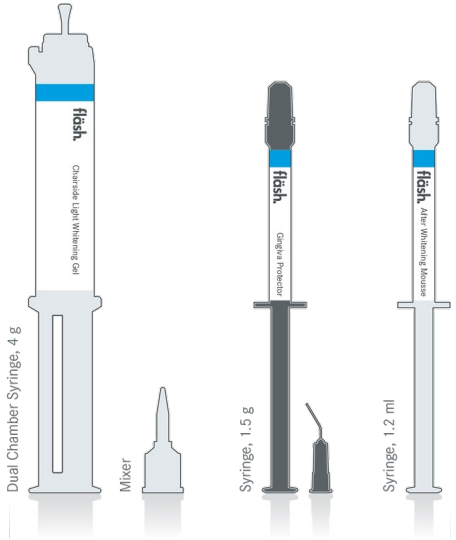
1 mm. Green. Neutral pH.

01 **Dual-chamber 32% HP.**
Mixed fresh, full potency until use.

02 **Green = precision.**
Every surface visibly covered.

03 **Oxidation, not erosion.**
Chromogens oxidised at neutral pH.

04 **Minimum 1 mm, everywhere.**
A thin layer leaves a pale patch.



The fläsh syringe set: gel, barrier, mousse



PHASE 3 · APPLY

The Lamp: Position Is Power

01 Lock the geometry.

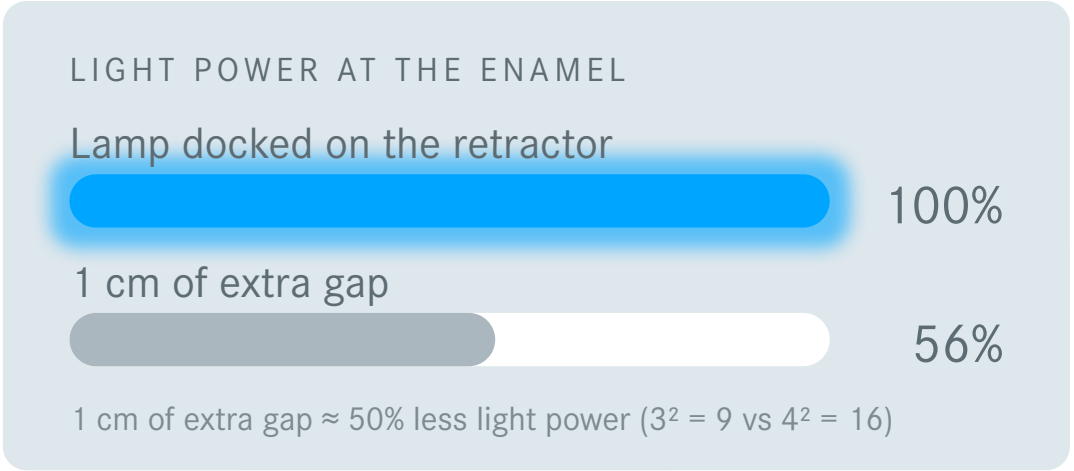
Patient at 45°, lamp head locked on the retractor — intensity level IIII.

02 The expensive centimetre.

One centimetre of gap costs ≈50% of the power.

03 Complementary-colour activation.

Blue 460 nm light, absorbed by the green gel — no UV, no infrared, below 39 °C.



EVIDENCE

Ontiveros & Paravina, J Dent 2009

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The cycle length is not negotiable – it is the protocol.

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PHASE 4 OF 5

Activate & Monitor

Insert the card – let the system lead your assistant.

PHASE 4 · ACTIVATE & MONITOR

The fläshcard Discipline

01 3–4 cycles × 15 minutes.

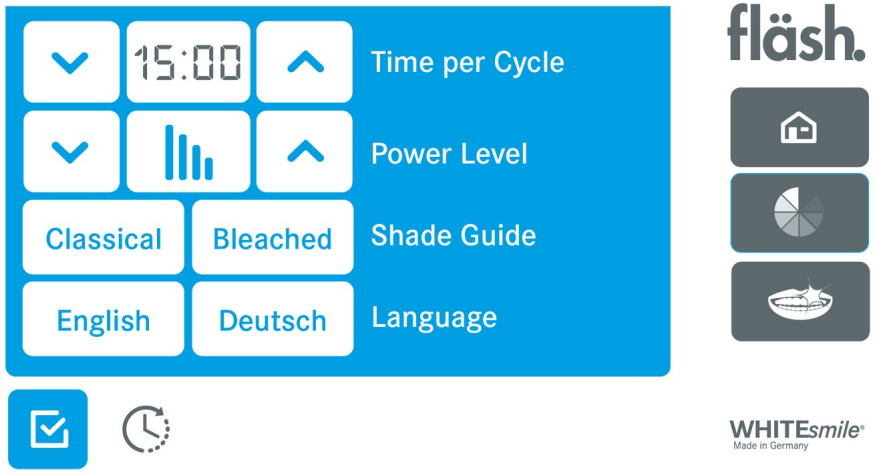
The cycle length is not a negotiable estimate — it is the protocol.

02 Fresh gel between cycles.

Suction off the spent gel — never let the patient rinse.

03 Closed, but flexible.

Every remaining minute is banked by the card — the hero of our mini case.



The touchscreen guides each cycle

PHASE 4 · ACTIVATE & MONITOR

Comfort Checkpoints

01 Agreed hand signal.

Established before the first cycle begins.

02 Verbal check.

At every gel change — ask, don't assume.

03 Pulse mode.

Cycles 2–3, at the first sign of sensitivity.

04 Never unattended — Sin #6.

Your assistant manages the human being.



CLINICAL FOCUS

"Effortless for the clinical team"

The system carries the clock and the physics.

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PHASE 5 OF 5

Seal & Instruct

The last ten minutes decide how the first sixty are remembered.

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PHASE 5 · SEAL & INSTRUCT

Finish Without Undoing It

01 No rinsing between cycles.

Rinsing destroys isolation — suction only.

02 Gel off, barrier last.

Protect the tissue until the chemistry is gone.

03 After Whitening Mousse.

Soothes and remineralizes the treated surface.

04 Final shade & after photos.

Value-ordered guide, shades gained, identical setup.

CLINICAL FOCUS

This is the moment

the mirror does your marketing.



PHASE 5 · SEAL & INSTRUCT

The Aftercare Window

Minutes	~2 h	18–24 h
Basal pellicle layer forms	Near-full thickness	Maturation in vivo

01 Mind the first hours.

No magic number — make chromogen-light choices while the pellicle re-establishes.

02 Pre-empt the rebound.

Slight settling over the next days is normal — say it in the chair.

EVIDENCE

Hannig M., Clin Oral Investig 1999



THE IN-OFFICE PROCEDURE · STEP BY STEP

Prepare & Protect



Insert the lip retractor



Air-dry teeth and gingiva



Apply the Gingiva Protector

Clean, isolate and seal the soft tissue before any gel.

THE IN-OFFICE PROCEDURE · STEP BY STEP

Prepare & Protect



Light-cure the Gingiva Protector



Check the seal



Every complication is prevented here — the barrier is the treatment.

THE IN-OFFICE PROCEDURE · STEP BY STEP

Apply & Activate



Apply the gel, 1 mm thick



Adjust the lamp, start 15 min



Suction off the gel — no rinsing

Repeat 3× (max 4×) — fresh gel each cycle, suction only, never rinse.



THE IN-OFFICE PROCEDURE · STEP BY STEP

Seal & Finish



Rinse off residual gel with water



Remove the Gingiva Protector



Apply After Whitening Mousse

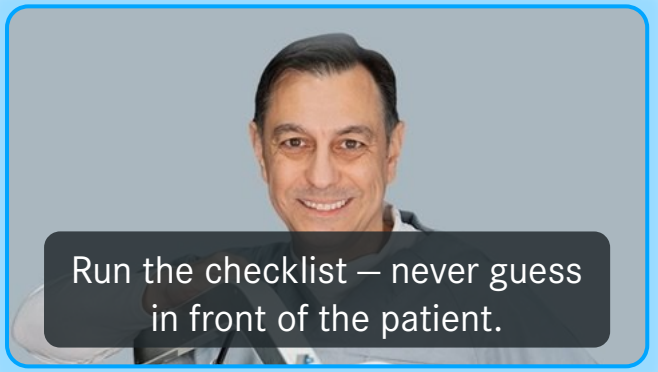
After Whitening Mousse soothes and remineralises — then final shade & photos.

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MINI CASE · DIAGNOSE

“The Uneven Case”



A 38-year-old at the end of cycle 3 – centrals magnificent, cervical thirds still yellow, canines two steps behind, chalky patches on the laterals. She is holding the mirror.

FINDING	CAUSE
Cervical thirds still yellow	➡ Barrier over-extended onto cervical enamel
Patchy brightness	➡ Gel layer below 1 mm in zones – Sin #5
Everything underperforming	➡ Lamp gap or tilt – 1 cm ≈ −50% power
Canines two steps behind	➡ Thicker dentin – biology, not error
Chalky white patches	➡ Transient dehydration – not damage

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MINI CASE · RESOLVE

The Fix & The Conversation

1 Re-isolate, correctly

Rebuild the barrier and confirm the retractor is docked on the lamp head.

2 Fresh 1 mm gel

Full-thickness layer on the canines and cervical thirds — the lagging zones.

3 Spend banked minutes

The cycle time the fläshcard saved becomes a targeted touch-up.

4 Say the words

“The canines are always last. The white areas are temporary.” Book the one-week photo.

KNOWLEDGE CHECK

Cervical thirds still yellow after cycle 3 — what is the most likely cause?

A Gel layer below 1 mm

B Barrier over-extended onto cervical enamel

C Lamp gap or tilt

KNOWLEDGE CHECK · ANSWER

Cervical thirds still yellow after cycle 3 — what is the most likely cause?

- A Gel layer below 1 mm
- **B Barrier over-extended onto cervical enamel**
- C Lamp gap or tilt

The barrier covered the cervical enamel — the gel never touched it. Re-isolate correctly, then apply fresh 1 mm gel.



SUMMARY + NEXT ACTION

The SOP Blueprint

Document
Clean first, baseline shade in value order, photograph like a scientist.

Protect
Vitamin E or vaseline, correct retractor, goggles, cured barrier — avoid the Seven Sins.

Apply
A minimum 1 mm of green gel, working by oxidation at neutral pH.

Activate & monitor
Lamp locked on the retractor · fläshcard discipline · comfort checkpoints.

Seal & instruct
Mousse, the aftercare window, the departure gift — identically, every time.



NEXT ACTION · THIS WEEK, NOT THIS QUARTER

Make It Your Team's Reflex

1. **Run one full dry run**
   No patient in the chair — your assistant drives the fläshcard start to finish.
2. **Appoint an SOP owner**
   One team member owns this protocol — and audits the clinic against it.
3. **Laminate the phases**
   Visible chairside, next to the Seven Sins of Improper Isolation.

UP NEXT

Episode 03 · Sensitivity Prevention & Management

Preventing it, managing it — and turning a complaint into loyalty.

